

For Yujie's parents — what the doctors have found, why it happened, and what can be done

(Written 19 August 2026, after the MRI scan of 18 August and the radiologist's report. Yujie has been referred to a neurosurgeon, Dr Jeffrey Brennan, in Sydney.)

The one-paragraph version

The things you have known about Yujie since she was small — the constipation and the bladder accidents that never quite went away, the left leg and foot being smaller and colder than the right, the mark on her lower back — and the pain she has had down her left leg this past year, **all turn out to have one single cause**. At the very bottom of her spinal cord, right where the spine meets the pelvis, there is a fatty lump that has been there since before she was born. It is holding the end of the spinal cord down when it should have been free to move. The doctors call this a **tethered spinal cord** caused by a **spinal lipoma** (a lipoma is a lump of fat). It is a condition she was born with. It is not dangerous in the way a cancer is — the lump is fat, not a tumour — but it does need a specialist, and it can be treated with an operation.

What it is, in plain words

The spinal cord is the thick nerve cable that runs from the brain down inside the backbone. In everyone, the cord itself stops about two-thirds of the way down the back; below that there are only fine nerve threads. Those lowest nerves are the ones that control **the bowel, the bladder, the private parts, and the small muscles of the feet**.

In Yujie, the bottom of the cord did not end where it should. It is stuck lower down, held by a lump of fat about 12 cm long that is attached to it. Because it is stuck, the cord has been gently but constantly stretched for her whole life — every time she grew, every time she bent. The nerves that were stretched are exactly the ones for the bowel, the bladder, the private parts and the left foot. That is why:

- she had trouble with constipation and holding urine as a child, and still does;
- her left leg and foot grew smaller, and the left foot has a high arch and curled toes — the small muscles of that foot never received full nerve signals while she was growing;
- her left leg has always been colder;
- in the last year the fatty lump has begun to press on two nerves going down her left leg, which is where the pain from her bottom to her heel comes from.

The mark on her lower back is part of the same thing. It is not an ordinary birthmark. It is a small tunnel in the skin — the doctors call it a **dermal sinus** — that formed at the same time as the fatty lump. **Please do not squeeze it, press on it, or put anything on it**. If it ever becomes red, swollen or leaks fluid, or if Yujie has a fever with a stiff neck or a bad headache, she must go to hospital the same day.

What caused it — and what did not

This formed **in the third and fourth week after conception** — before her mother would even have known she was pregnant. At that stage the skin and the spinal cord are one layer, and they have to separate from each other. In Yujie, at one small spot at the bottom of the back, they separated slightly wrongly. A little tissue got in where it should not have, and turned into fat that stuck to the cord. That is the whole story.

We want to say the following as clearly as we can, because we know how parents think:

- **Nothing you did or did not do caused this**. Not diet, not medicine, not vitamins, not work, not anything during the pregnancy or after. Doctors do not know why it happens in one baby out of many thousands; it happens in families of every kind and every country.
- **You could not have known**. The mark on her back looks exactly like a birthmark — that is what everyone, including most doctors, would have called it. Constipation and wetting in a small child are so common that no one would have thought of the spinal cord. This condition is often only found in a person's twenties or thirties, when the pain starts, precisely because it hides so well. Yujie is 29 and it has just been found; that is the normal way this story goes.
- **You did the right things**. You managed the constipation, you managed the accidents, you got her through school and university and into her working life. That care mattered.

What can be done

1. **A neurosurgeon will see her**. Her doctor has already referred her to Dr Jeffrey Brennan, a spinal neurosurgeon at Royal Prince Alfred Hospital in Sydney. He will examine her, look at the scan in detail, and explain the options.
2. **The usual treatment is an operation** to release the cord — the surgeon opens the lower back, carefully separates the fat from the cord and cuts the tether, and removes the small skin tunnel. It is a well-known, well-practised operation, done in a hospital, usually with a few days' stay and some weeks of recovery. It is not emergency surgery; it will be planned properly.
3. **What the operation is meant to do**: stop things getting worse — the pain, the risk of more weakness, the recent changes in her bowel. Pain usually improves. **What we should not expect**: that everything she has lived with since childhood will disappear. Bladder and bowel function sometimes improve, more often stay as they are but stop worsening. The foot will stay as it is. She will still need to manage her bowel and bladder afterwards, with better help now that everyone understands the cause.
4. **She can still have children**. This condition does not affect fertility. When the time comes, her pregnancy will need a little extra planning (the anaesthetist needs to know about her back, and she should take a higher dose of folic acid beforehand), but women with this condition have healthy pregnancies and healthy babies.

What happens next

- Appointment with the neurosurgeon (being arranged now).
- Copies of the scan being collected.
- Yujie has a written list of warning signs (new numbness around the bottom, sudden trouble passing urine, sudden weakness in a leg, fever with stiff neck, the back mark becoming red or leaking) — any of these means hospital the same day, and Shane knows this too.

The main thing

After nearly thirty years, there is finally **one explanation for everything**, it is **not her fault and not yours**, and it is **something surgeons know how to treat**. That is good news, even though it does not feel that way at first.